

The Financial Opportunity Cost of Continued Navy Service for Primary-Care Physicians

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Purpose and Economic Framework

The U.S. Navy Medical Corps must understand the alternatives physicians face when deciding whether to remain after their initial obligation. Using a board-certified general internist in San Diego, this paper asks: How much financial compensation does a physician forgo by remaining in the Navy for four more years rather than accepting a feasible civilian position? The U.S. Government Accountability Office (GAO, 2020) found that maximum military cash compensation trailed civilian medians in 21 of 27 medical and dental specialties reviewed and recommended better wage, bonus, and replacement-cost data. Those findings motivate this case study but do not prove a current Navy primary-care pay gap.

Opportunity cost is the value of the best forgone alternative. Here it is measured as the present value of civilian cash compensation minus Navy cash compensation over the same period. Prior medical-school support is sunk; future pay, obligations, retirement accrual, and working conditions remain relevant (Frank et al., 2022). Mission, leadership, teaching, operational medicine, assignment control, workload, and family stability may also create compensating wage differentials. Therefore, the cash gap is not a complete valuation of either career and does not identify the bonus required for retention.

Method

The base case is an O-4 general internist just past 10 active-service years, with dependents, assigned to ZIP code 92134. The horizon is 2027-2030 in constant 2026 dollars. Rank remains O-4; basic pay advances from the over-10 to the over-12 rate. The analysis uses a 5% real discount rate and year-end payments. The civilian comparator is Marit Health's published San Diego internist average of \$459,057, held constant in real terms (Marit Health, 2026). Because this is a mixed-employer, self-reported benchmark rather than a matched offer, the result remains conditional on its comparability.

Compensation Inputs

Appendix A, Table 1 itemizes Navy cash pay and the conditional retention bonus.

Each annual gap is discounted as $(\text{civilian cash} - \text{Navy cash})/(1.05)^t$, where $t = 1, 2, 3, 4$.

BAH and BAS receive federal income-tax exclusions, so equal gross cash would not imply equal take-home pay (Internal Revenue Service, 2025). A full comparison would also include taxes, health coverage, employer retirement contributions, malpractice costs, and transition expenses.

Pension Limitation

The base case ends at 14 years, before the usual 20-year active-duty retirement threshold, so no pension payment appears in the four-year cash table. This omits an important career-stage effect. Conditional on completing service to 20 years, the present value of the modeled pension increases as eligibility approaches; overall incentives also depend on civilian earnings and other circumstances. For illustration, assuming a \$120,000 High-3 basic-pay base, 30 constant real payments, and a 5% discount rate, a \$48,000 BRS pension has a conditional present value of about \$453,000 at 10 years, \$669,000 at 18 years, and \$703,000 at 19 years. These values cannot simply be subtracted from the four-year cash gap because the horizons and assumptions differ (Department of Defense, n.d.).

Results

Figure 1 shows the annual comparison and cumulative gap. The base case produces an undiscounted cash gap in constant 2026 dollars of \$717,126 and a present-value gap of approximately \$636,209. At 3%, the present-value gap is \$666,715; at 7%, it is \$607,907. Without the assumed \$48,000 retention agreement, the four-year present-value gap rises by approximately \$170,206. These estimates compare compensation streams; they do not predict separation. Dahle (2021) illustrates substantial within-specialty pay variation; his examples do not establish the distribution of San Diego internist offers. Illustrative annual civilian salaries of \$350,000 and \$400,000 reduce the four-year cash gap to about \$249,500 and \$426,800, respectively. These are sensitivity assumptions, not verified offers. A smaller matched gap makes other benefits more consequential; a larger gap strengthens the case for compensation changes.

Policy Implications

The Navy should first test a limited communication pilot while pursuing bonus advocacy. My rationale is administrative feasibility: changing authorized bonus rates requires a formal pay-policy and funding process, whereas a bounded communication effort may be more feasible through existing channels. This does not mean Navy Medicine has no influence over pay or that communication can close the cash gap. Its value would be correcting misunderstandings about real, accessible opportunities and existing compensation, not substituting advertising for inadequate pay.

A proposed design would enroll physicians whose obligations end within 12 months and randomly assign commands to enhanced communication or standard information, balancing specialty and years of service. Measure awareness at baseline and six months, agreement acceptance over 12 months, and actual retention through 24 months, accounting for communication spillover. If awareness improves without a meaningful retention gain at

acceptable cost, stop expansion and prioritize compensation or working conditions.

Inconclusive results require more evidence, not a claim of success.

Additional Options

Team-based staffing could shift appropriate work toward nurse practitioners and physician assistants while preserving physician capacity for complex care and readiness. VA evidence supports feasibility in selected populations but not one-for-one Navy substitution (Liu et al., 2020; Morgan et al., 2019). The Navy should also address workload, administrative burden, leadership support, deployment, and work-life concerns associated with burnout and turnover-related outcomes (Vie et al., 2024; Wilk et al., 2023).

Limitations

The model uses one rank, location, and civilian benchmark. It assumes bonus eligibility and excludes promotion, taxes, health coverage, retirement contributions, malpractice costs, moonlighting constraints, and probability-adjusted pension value. It does not estimate Navy attrition, replacement costs, retention elasticity, or readiness losses. Future analysis should compare matched career paths by specialty and service stage.

Conclusion

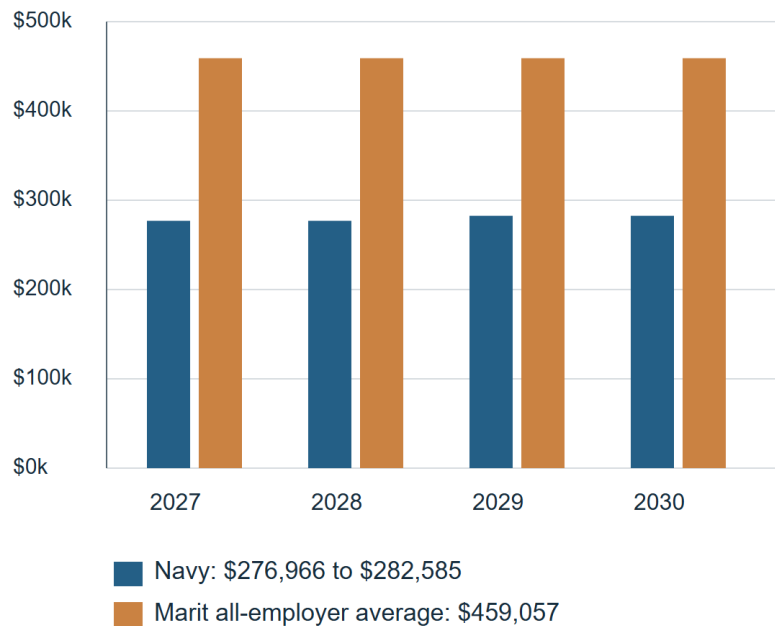
For the modeled O-4 internist, the four-year present-value gross-cash gap is approximately \$636,209 at 5%. The estimate is substantial but incomplete. Conditional on completing service to 20 years, the present value of the modeled pension increases as eligibility approaches; overall incentives also depend on civilian earnings and other circumstances. The Navy should test targeted communication, evaluate bonuses by marginal retention effect, improve working conditions, and collect better career-stage data.

Figure 1. Annual compensation and cumulative discounted cash gap

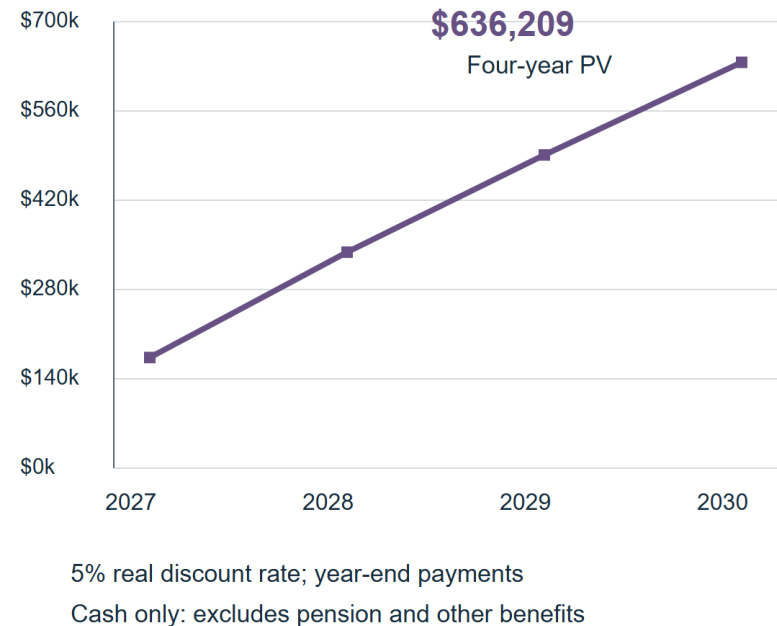
Internal medicine in San Diego

Illustrative Navy O-4 versus published local benchmark | Constant 2026 dollars

A Annual gross cash



B Cumulative discounted cash gap



Navy: O-4, 10 starting years, dependents, ZIP 92134; conditional \$48,000 annual RB.

Sources: DFAS; documented BAH input; Marit Health (April 2026). Not a matched civilian offer.

Illustrative O-4 scenario; 5% real discounting. The civilian series is not a matched offer.

Pension, taxes, and other benefit differences are excluded.

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Appendix A. Compensation inputs

Navy component	2027–2028	2029–2030
Basic pay	\$113,040	\$118,660
BAH with dependents	\$60,984	\$60,984
BAS	\$3,942	\$3,942
Incentive pay	\$43,000	\$43,000
Board-certification pay	\$8,000	\$8,000
Assumed four-year retention bonus	\$48,000	\$48,000
Total Navy gross cash	\$276,966	\$282,585

Table 1. Base-case cash compensation. The retention bonus is conditional on eligibility and an executed agreement. Totals use unrounded inputs (Defense Finance and Accounting Service [DFAS], 2026a, 2026b, 2026c; Defense Travel Management Office, n.d.).